suspect that eyes with more severe or concerning presentations were more likely to be offered PPV. Additionally, many of these patients were systemically ill, which may have delayed PPV if active infection or severity of illness prevented them from undergoing surgery or may have prevented their recovery, potentially impacting VA outcomes.

To our knowledge, this is the largest and only multicenter case series on EE associated with IMD. The limitations of this study include its retrospective nature and small sample size. Additionally, the short duration of follow-up data available for these patients may skew VA outcomes. In conclusion, EE associated with IMD causes poor VA outcomes despite treatment. Bacterial pathogens caused more severe disease compared with fungal infections. Future larger studies are needed to determine optimal management for this patient population.

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Conception and design: Ciociola, Fekrat, Greven
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Abbreviations and Acronyms:
EE = endogenous endophthalmitis; **CI** = confidence interval; **IMD** = indwelling medical device; **logMAR** = logarithm of minimal angle of resolution; **PPV** = pars plana vitrectomy; **VA** = visual acuity.

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Uveal Melanoma and the IRIS® Registry (Intelligent Research in Sight): A Pilot Analysis and Future Directions

Uveal melanoma (UM), although the most common primary intraocular malignancy in adults, remains a rare condition with incidence of ∼ 5 to 6 people per million per year.[1] The IRIS® Registry (Intelligent Research in Sight) is the nation's largest electronic health record (EHR)-based ophthalmology registry. It collects information in an automated fashion, in contrast to many other clinical databases that typically require manual data entry.[2] Given the Registry's potential in evaluating rare diseases, we sought to identify and characterize patients with UM within the IRIS Registry to determine its feasibility as a tool to study UM.

The methods of the IRIS Registry have been previously described.[2] The query was performed on September 6, 2019. Patients with UM diagnosed between 2013 and 2017 were identified using the International Classification of Disease 9 and 10 edition codes (Table S1, available at www.ophthalmologyretina.org). Information regarding tumors, clinical examination, treatments, and visual outcomes were obtained. The date at which a patient was given a qualifying diagnosis was defined as the index date. Data elements of interest were compared before UM diagnosis (i.e., preindex) and at or up to 1 year after UM diagnosis (postindex).

Overall, 16 278 patients with diagnostic codes consistent with UM were identified. After excluding 1699 patients with unspecified and bilateral UM, 241 patients with choroidal metastases, 65 patients with malignant neoplasm of the retina, and 38 patients